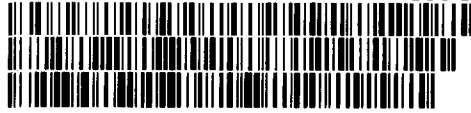


LABORATORY MEDICINE PROGRAM

UUID: C5F78F74-2FDB-4D1C-8133-410918A35A64

TCGA-EM-A3FM-01A-PR

Redacted



Surgical Pathology Consultation Report

Patient Name:

MRN:

DOB:

Gender:

HCN:

Ordering MD:

Copy To:

Service:

Visit #:

Location:

Facility:

Accession #:

Collected:

Received:

Reported:

1 CD-0-3

carcinoma, papillary, thyroid 8260/3

Site: thyroid, NOS C73.9

fw
11/22/11

Specimen(s) Received

1. Nck: Right neck content single stitch superior double stitch anterior
2. Lymph node: R paratracheal node
3. Lymph node: Delphi lymphnode
4. Thyroid: total thyroid stitch mark L upper pole (fresh for tumor banking)

Diagnosis

1. Metastatic classical variant papillary thyroid carcinoma: Lymph nodes, 7 of 41, (right neck) dissection specimen
2. Metastatic follicular variant papillary thyroid carcinoma: Lymph node (right paratracheal) excisional biopsy
3. Metastatic classical variant papillary thyroid carcinoma: Lymph node, 1 of 3, (Delphian node) excisional biopsy
4. Multifocal papillary carcinoma, dominant widely invasive classical variant with focal tall cell change (5%); follicular nodular disease and mild reactive C-cell hyperplasia: Thyroid
Metastatic classical variant papillary thyroid carcinoma, dominant with focal extranodal extension: Lymph nodes, 4, right perithyroidal
Metastatic follicular variant papillary thyroid carcinoma: Lymph nodes, 3, right perithyroidal
Metastatic follicular variant papillary thyroid carcinoma: Lymph node, isthmic
No pathological diagnosis: Thymus, right perithyroidal
Simple epithelial cyst: Parathyroid, isthmic
- Total thyroidectomy specimen

Comment

The dominant lesion is a widely invasive classical variant papillary carcinoma with focal tall cell change (5%). The second dominant nodule is a mixed follicular and classical variant papillary carcinoma. There is also follicular nodular disease and mild reactive C-cell hyperplasia.

Synoptic Data

Procedure:

Received:

Specimen Integrity:

Specimen Size:

Other: Total thyroidectomy with Delphian, right paratracheal and right neck dissection

Fresh

Intact

Right lobe:

5.7 cm

3.0 cm

2.1 cm

Left lobe:

Criteria	Yes	No
Diagnosis Discrepancy		X
Primary Tumor Site Discrepancy		X
HIPAA Discrepancy		X
Prior Malignancy History		X
Dual/Synchronous Primary Noted		X
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	[Signature]	
Date Reviewed:	11/22/11	

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3.3 cm
2.2 cm
0.8 cm
Isthmus +/- pyramidal lobe:
2.1 cm
1.8 cm
0.8 cm
Additional specimen: right neck dissection
12.5 cm
4.0 cm
1.0 cm

Specimen Weight:

30.4 g

Tumor Focality:

Multifocal, bilateral

Multifocal, midline (isthmus)

----- DOMINANT TUMOR -----

Tumor Laterality:

Right lobe

Tumor Size:

Greatest dimension: 2.2cm

Additional dimension: 2.0 cm

Additional dimension: 0.9 cm

Histologic Type:

Papillary carcinoma, classical (usual)

Papillary carcinoma, other variant: with focal tall cell change (5%)

Classical (papillary) architecture

Classical cytomorphology

Tall cell cytomorphology

Histologic Grade:

Not applicable

Margins:

Margins uninvolved by carcinoma

Tumor Capsule:

None

Tumor Capsular Invasion:

Present, extent widely invasive

Lymph-Vascular Invasion:

Indeterminate

Perineural Invasion:

Indeterminate

Extrathyroidal Extension:

Not identified

----- SECOND TUMOR -----

Tumor Laterality:

Right lobe

Tumor Size:

Greatest dimension: 1.2cm

Histologic Type:

Papillary carcinoma, classical (usual)

Papillary carcinoma, follicular variant

Classical (papillary) architecture

Follicular architecture

Classical cytomorphology

Histologic Grade:

Not applicable

Margins:

Margins uninvolved by carcinoma

Tumor Capsule:

None

Tumor Capsular Invasion:

Present, extent minimal

Lymph-Vascular Invasion:

Not identified

Perineural Invasion:

Not identified

Extrathyroidal Extension:

Not identified

TNM Descriptors:

m (multiple primary tumors)

Primary Tumor (pT):

pT2: Tumor more than 2 cm, but not more than 4 cm, limited to thyroid

Regional Lymph Nodes (pN):

pN1b: Metastases to unilateral, bilateral or contralateral cervical or superior mediastinal lymph nodes

Number of regional lymph nodes examined: 53

Number of regional lymph nodes involved: 17

Lymph Node, Extranodal Extension Present

Not applicable

Distant Metastasis (pM):

Additional Pathologic Findings:

Adenomatoid nodule(s) or Nodular follicular disease

Parathyroid gland(s), other: simple epithelial cyst

C-cell hyperplasia

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Other: additional multiple papillary microcarcinomas, isthmus: 0.7 cm, classical variant, and left lobe: 0.7 cm, encapsulated follicular variant with focal capsular invasion; 0.4 cm, classical variant; 0.1 cm, follicular variant and 0.1 cm, classical variant.

*Pathologic Staging is based on AJCC/UICC TNM,

Electronically verified by:

Clinical History

Papillary thyroid carcinoma

Gross Description

1. The specimen labeled with the patient's name and as "Right neck contents single stitch superior double anterior" and consists of a functional neck dissection, oriented as indicated, and measures 12.5 cm FI by 4.0 cm AP by 1.0 cm ML. Sternocleidomastoid muscle, internal jugular vein and submandibular gland are not received with the specimen. Multiple lymph nodes are identified within the specimen. These range in size from 0.1 to 2.1 cm. Representative sections, lymph nodes submitted in toto:

Superior third

1A-C multiple lymph nodes, per block

1D-F one lymph node bisected, per block

Middle third

1G-H multiple lymph nodes, per block

1I one lymph node bisected

Lower third

1J multiple lymph nodes

Posterior cervical triangle

1K multiple lymph nodes

2. The specimen labeled with the patient's name and as "Right paratracheal node" and consists of a lymph node that measures 1.3 x 0.8 to 1.2 cm. There are tiny fragments of adherent adipose tissue. Submitted in toto:

2A one lymph node, bisected

3. The specimen labeled with the patient's name and as "Delphian lymph node" and consists of a lymph node that measures 1.0 x 0.7 x 0.5 cm. There are adherent fragments of adipose tissue. Submitted in toto:

3A one lymph node, bisected

4. The specimen labeled with the patient's name and as "Total thyroid stitch marks L. upper pole" consists of a thyroid gland that is received oriented with a suture and weighs 30.4 g. The right lobe measures 5.7 cm SI x 3.0 cm ML x 2.1 cm AP, the left lobe measures 3.3 cm SI x 2.2 cm ML x 0.8 cm AP and the isthmus measures 2.1 cm SI x 1.8 cm ML x 0.8 cm AP. The external surfaces have fibrous adhesions and are painted with silver nitrate. No parathyroid glands are identified grossly. There is a portion of soft tissue that extends along the base of the specimen from the right lobe to the left lateral aspect of the isthmus. Within this are multiple nodules that are consistent with lymph nodes ranging from 0.3 to 1.4 cm. The right upper lobe contains 1 delineated nodule that measures 2.0 cm SI x 2.2 cm ML x 0.9 cm AP. The remainder of the thyroid shows additional nodules that range from tan and translucent to fleshy and yellow. Sections of the main nodule and normal tissue are stored frozen. Representative sections of the remainder of the specimen are submitted as follows:

4A-C every other section of main nodule

4D-L remainder of right lobe, superior to inferior

4M-O isthmus

4P-W left lobe, superior to inferior

4X multiple lymph nodes underlying right lobe

4Y largest lymph node underlying right lobe, bisected

4Z 2 possible lymph nodes underlying isthmus

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